STRATEGIC MANAGEMENT

Thyrocare: Disrupting the Indian Diagnostic Industry; Suresh Srinivasan, Swati Agarwal, Sameera Sepuri; Product Number:9B20M09; 6 May 2020, 20 pages, Source: Ivey (Award-Winning Case; Ivey; 2nd position Globally in 2018)

Naman Jain

Section – 2

FT272022

Initial prompt

I am an MBA student working on a Strategic Management case set in the Indian diagnostics industry. I have read the case and prepared my own summary and preliminary recommendation below

For this discussion, I will call the company "A Diagnostics" and its founder and CEO "Mr. B." These are dummy names. Please do not try to identify the actual company or use outside information about it. The case is set in July 2017, and the analysis must remain within that time period.

I do not want you to solve the case immediately. I want you to act as a strategy companion who questions my reasoning and helps me see what I may have missed. Use only the facts I provide here. If an important fact is missing, point it out instead of assuming it.

A Diagnostics operates in the Indian medical diagnostics industry. In July 2017, Mr. B was reviewing the performance after the company reported its results for the financial year ended March 2017.

The company was in a strong position. Its revenue had increased by about 28% despite a slowdown in the wider economy. It also had one of the highest EBITDA and profit margins in the industry. Its 2016 public issue had been subscribed about 75 times, and its market value had reached roughly Rs. 33.45 billion.

The strong performance did not make the strategic choice simple. Mr. B had to decide where the next phase of growth should come from. One option was to keep expanding the company's pathology and preventive testing business. The other was to invest in new areas such as PET-CT imaging, tuberculosis testing, water testing, wellness clinics and home testing.

There was also a longer term concern. The company had built its success around low prices, centralized processing and high test volumes. Mr. B had to judge whether competitors could copy this model and reduce the advantage A Diagnostics currently enjoyed

The industry situation

The Indian diagnostics market was large, growing and highly fragmented. It was estimated to be worth about Rs. 77 billion in 2017.

Around 37% of the market was served by unorganized providers. Hospital laboratories, standalone centres and regional diagnostic businesses accounted for another large share. National diagnostic chains together controlled only about 6% to 7% of the market. A Diagnostics itself had a market share of about 0.8%, while two larger national competitors had shares of roghly 2.4% and 2.5%.

Pathology accounted for about 56% of the diagnostics market, while imaging accounted for approximately 44%. Preventive care represented only around 9% of the total market. Most diagnostic spending still related to sick care.

Preventive testing nevertheless appeared to have considerable room to grow. Consumers were becoming more aware of lifestyle diseases and early detection. Rising incomes, insurance coverage and digital access could also increase demand.

The industry had some structural problems. Quality varied across laboratories, and relatively few facilities had recognized accreditation. Customers chose from many providers, and switching costs were low. Most routine tests were standardized, which made price comparison easy. Doctors, hospitals, corporate clients and collection partners influenced where samples were sent. Online aggregators could make prices still more transparent

Diagnostic laboratories also carried substantial fixed costs. They needed equipment, trained staff, facilities, information systems and quality controls. The case gives an indicative industry cost structure in which consumables were about 25% of revenue, staff costs 20%, partner commissions 10%, rent 4%, other overheads 14%, depreciation 5%, finance costs 1% and taxes 6%. This left an indicative net profit margin of around 15%.

Because a large part of the cost base was fixed, volume mattered. Once the required infrastructure was in place, each additional test could lower the average cost per test.

How A Diagnostics was different

Mr. B started the company in 1996 with about Rs. 200,000. He came from a farming family with limited financial resources and had earlier worked as a scientist at a government atomic research institution.

His personal habits shaped the company. He believed in simple operations, low overheads and close control of expenditure. The organization remained lean and avoided positions or facilities that management did not consider necessary. This culture seems to have supported the company's low-cost strategy.

The company initially concentrated on thyroid testing and later expanded into biochemistry and preventive health packages. It did not copy the usual diagnostic chain model of operating a large number of full-service laboratories.

Instead, collection partners obtained samples from patients. The samples moved through hubs and regional laboratories before reaching the Central Processing Laboratory in Navi Mumbai. The company then sent the test results electronically to the collection partners and customers.

This arrangement allowed the partners to manage much of the local collection activity while A Diagnostics concentrated on processing. It kept the front end relatively light and combined a large number of samples at a smaller number of processing facilities.

By 2017, the company had one central laboratory and six regional processing laboratories. Its wider network included about 1,041 authorized service providers, 535 service providers and 687 aggregators. It processed approximately 14.3 million samples and 76.4 million individual tests during the year with around 850 employees.

The company had also invested in automation. In 2014, it installed an automated track of about 300 feet at its central laboratory. Barcodes and connected systems helped move samples through the testing process with limited manual handling. Reports were generated and delivered digitally.

Higher volumes gave the company greater bargaining power when purchasing reagents, consumables and equipment. It could negotiate lower prices and more favourable commercial arrangements than a small local laboratory.

A Diagnostics offered relatively low and transparent prices. According to the case, it had avoided increasing prices for about 20 years while continuing to earn strong profits. This suggests that it earned its margins by reducing cost per test rather than by charging customers a premium.

Capacity and the meaning of scale

The 6 regional laboratories could process about 44,000 samples/day but were handling only around 24,000/day. Their combined capacity utilization was therefore close to 55%.

Utilization was uneven like New Delhi was operating at about 80% and Hyderabad at 67%. Coimbatore was near 50%, Bhopal around 42%, and Kolkata and Bengaluru about 40% each.

This is important because scaling the core business does not require the company to invent new blood tests. It can scale by bringing more samples into the same system.

Possible sources of additional volume include more collection partners in underserved cities, home collection, online booking, corporate health programmes, hospital contracts and outsourced processing for smaller laboratories. The company can also sell preventive packages that include several tests, encourage customers to test at regular intervals and improve its presence in Tier 2 and Tier 3 cities.

If the existing laboratories process more samples without a similar increase in fixed costs, the average cost per test should fall. The unused capacity therefore appears to offer a lower risk growth opportunity than immediately building new facilities or entering unrelated businesses.

What the financial figures suggest

In FY2017, A Diagnostics reported revenue of about Rs. 3,158 million. The two larger competitors reported approximately Rs. 9,315 million and Rs. 9,519 million.

A Diagnostics was much smaller by revenue, but its profitability was unusually high. It recorded EBITDA of about Rs. 1,274 million and profit after tax of roughly Rs. 790 million. Its estimated PAT margin was close to 25%.

The comparable PAT margins of the two larger competitors were approximately 17% and 10%. They also employed about 3,746 and 6,000 people respectively, compared with only about 850 employees at A Diagnostics.

The company's revenue rose from around Rs. 1,911 million in FY2015 to Rs. 2,475 million in FY2016 and Rs. 3,158 million in FY2017. Profit after tax increased from about Rs. 452 million to Rs. 518 million and then to approximately Rs. 790 million.

My reading is that these figures support the claim that A Diagnostics had a more efficient operating model. Its advantage did not come from having the largest market share. It came from processing a large volume of tests with fewer employees and a concentrated laboratory network.

The new business options

PET-CT imaging could provide high revenue per procedure and support cancer diagnosis. However, it requires expensive scanners, cyclotron facilities, specialist staff and suitable locations. A pathology sample can be sent to a central laboratory, but a patient normally has to travel to an imaging centre. The economics therefore depend heavily on utilization at each location.

Tuberculosis testing appears closer to the existing business because it still involves sample collection, laboratory processing and reporting. Even so, it may require different technology, institutional relationships and dealings with government health programmes.

Water testing uses some of the same laboratory and process control skills, but the customers are different. They may include municipal authorities, factories, companies and other institutions rather than individual pathology customers.

Wellness clinics could give the company a direct relationship with consumers. They could offer preventive check-ups, consultations and health monitoring. The concern is that owned clinics would add rent, medical staff and customer service costs. They might also compete with the collection partners on whom the existing model depends.

My interpretation

I currently see A Diagnostics as a cost leader rather than a company competing through premium differentiation.

Its advantage does not appear to come from one rare machine or one test. It comes from the way several activities work together: centralized processing, automation, sample logistics, partner collection, purchasing volume, digital reporting and strict cost control.

A competitor could buy similar equipment or lower its prices. It would be harder to reproduce the whole system quickly because that would require sufficient sample volume, reliable logistics, partner relationships, purchasing power and an organization willing to operate with the same cost discipline.

I am less certain about how much of this advantage depends on Mr. B personally. His approach created the culture, but a founder-dependent organization may find it difficult to manage several new businesses at the same time.

My preliminary recommendation

I think A Diagnostics should give first priority to its pathology and preventive testing business.

There is still room to add collection partners, reach smaller cities, increase home collection, win corporate and hospital contracts and raise the utilization of the existing regional laboratories. These moves use capabilities the company already has and may require less capital than large scale diversification.

I would not reject every new business. I would test them separately and avoid committing substantial capital before their economics are clear.

TB testing appears to have the closest fit with the existing system. Water testing may be worth a limited institutional pilot, but management should treat it as a different market. Wellness clinics could be tested through partners or franchises instead of a large owned network. PET-CT should expand only in locations where demand, specialist availability, regulation and expected utilizaion can justify the investment.

My concern is that rapid expansion into imaging and owned clinics could make the company more capital intensive and harder to manage. It could weaken the cost structure that created its success.

Questions I need to answer

1. How did A Diagnostics disrupt the Indian diagnostic industry?
2. Define as to, what its strategy has been?

How I wnt you to work with me

Use the following strategy concepts where they genuinely help the analysis:

- industry attractiveness and organizational capability;
- Porter's Five Forces;
- Structure, Conduct and Performance;
- cost leadership and differentiation;
- economies of scale and capacity utilization;
- the Resource Based View and VRIN or VRIO;
- activity system fit;
- related and unrelated diversification;
- organizational capability and implementation risk.

Do not force every framework into the answer. Tell me when a framework adds little value or when two frameworks lead to the same conclusion.

Take me through no more than three iterations.

In the first iteration, review the context I have written. Identify missing facts, weak links and places where I have mixed a fact with an assumption. Ask me no more than three questions. Do not give me the final recommendation yet.

In the second iteration, challenge my interpretation after I answer your questions. Give me the strongest case for my position and the strongest case against it. Then ask up to two further questions that would help me improve the analysis.

In the third and final iteration, test my recommendations.

Keep facts, assumptions and interpretations separate throughout the discussion. Do not add information from later events or from outside the case. If the evidence cannot support a definite conclusion, say so.

Start with the first iteration only.

Iteration 1

AI response

This is a strong write-up — the capacity-utilization argument and the three-way financial comparison are both doing real analytical work, not just restating numbers. Three questions, per your revised cap:

3. The "no price increase for 20 years" claim.

You use this to conclude the company "earned its margins by reducing cost per test rather than by charging customers a premium" — a fairly load-bearing inference, since it's central to your cost-leadership framing. Is the 20-year no-price-increase detail something the case states directly, or is it something you're inferring from the company's general low-price positioning? I'm not asking you to defend it — just to mark whether this line belongs in "facts" or "interpretation" before we build further on it.

4. The network numbers don't add up as written.

You state the company had "1,041 authorized service providers, 535 service providers and 687 aggregators." But 535 + 687 = 1,222, not 1,041 — so either the 1,041 is a separate total that doesn't sum from those two categories, or one of the three figures is transcribed incorrectly. Can you check this against the case directly? This matters because "size of partner network" is one of your VRIN-adjacent arguments (partner relationships as a hard-to-replicate resource), so getting the actual structure right will matter in iteration 2.

5. Are the "possible sources of additional volume" things the case describes the company doing/considering, or your own list of plausible growth levers?

You list home collection, corporate programs, hospital contracts, outsourced processing for smaller labs, and Tier 2/3 expansion as ways to raise utilization. This list is doing a lot of work — it's the direct evidentiary base for your entire recommendation (prioritize organic capacity utilization over diversification). If the case explicitly mentions any of these as options management was considering, that's a much stronger foundation than if this is your own brainstorm of "things a diagnostics company could plausibly do." Which is it, and can you tell me which specific items (if any) come from the case text itself?

Answer what you can. Where the case is genuinely silent — say so rather than filling the gap; that's useful information too, and per your own instructions I won't be assuming anything on your behalf here.

Response to iteration 1

The low-price claim

The case directly states that the company had offered low prices for about 20 years while remaining the most profitable company in the industry. It does not say that every test price remained unchanged during this period. My earlier wording was therefore too strong.

I would treat the company's low-price history as a fact. The conclusion that it protected its margins by reducing the cost per test is my interpretation. The case supports this interpretation through evidence about centralized processing, automation, purchasing scale and high test volumes.

I would revise the sentence to:

"The company had maintained low prices for about two decades while remaining highly profitable. This suggests that its margins came mainly from a lower operating cost per test rather than premium pricing."

The network figures

My draft contains a transcription error. The case reports 1,041 Authorized Service Providers, described as comprising 357 service providers and 687 aggregators. I had incorrectly written 535 service providers.

There is also a small inconsistency in the case because 357+687=1,044, not 1,041. I should report the figures as given and acknowledge the difference rather than correct the source myself.

I would revise the sentence to:

"According to the case, the company had 1,041 Authorized Service Providers in 2017, described as comprising 357 service providers and 687 aggregators. The two reported subcategories total 1,044, which appears to be a minor inconsistency in the case."

I would also avoid claiming that the network is difficult to copy based on its size alone. Its strategic value depends on the relationships with partners, the volume of samples they generate and the logistics system connecting them to the laboratories.

The proposed sources of additional volume

This section combines facts from the case with my own recommendations.

The case directly states that preventive care was underpenetrated, involved repeat customers and already generated more than half of the company's revenue. It also describes a partner-based collection network, outsourced processing for smaller laboratories, digital systems and unused capacity in the

regional laboratories. These facts support the possibility of increasing volume within the existing business.

The case does not explicitly present all the following as management proposals:

- adding partners specifically in underserved cities;
- expanding online booking as a formal growth programme;
- securing new corporate health programmes;
- winning additional hospital contracts;
- targeting Tier 2 and Tier 3 cities.

These are my suggestions based on the company's operating model. I should label them as recommendations rather than facts from the case.

I would revise the paragraph to:

"The case shows that preventive testing, repeat customers, partner-based collection, outsourced laboratory processing and unused regional capacity could support further growth in the core business. Based on these facts, I believe the company could increase collection density, improve digital access, pursue selected institutional relationships and extend its geographic reach. These are my proposed growth options rather than alternatives explicitly listed by management in the case."

Iteration 2

AI response

Both revisions are careful and well-evidenced — you've resolved the tension I raised on founder dependency in exactly the right way: not by declaring it settled, but by separating what's institutionalized (routines, technology, partner network, a real org structure) from what remains genuinely unproven (whether cost discipline survives a leadership transition). That's a defensible, case-grounded position rather than a guess in either direction.

On competitor imitability, your narrower conclusion is the right calibration too — "could probably imitate individual activities and prices, but replicating the complete cost structure would require substantial changes to existing operating models" is a claim the case evidence actually supports, versus the stronger "cannot be imitated" claim you might have been tempted toward.

Iteration 2 — Strongest case for your position, strongest case against it

The strongest case for your interpretation (cost leadership via activity-system fit):

Your argument is that no single resource explains the advantage — it's the combination: centralized processing + automation + partner-based collection + purchasing scale + digital reporting + cost discipline, working together. This is a genuinely strong reading, and the financial evidence backs it unusually well for a case at this level: A Diagnostics runs at roughly a quarter of the revenue of its two nearest competitors but converts that into a higher PAT margin (≈25% vs. ≈17% and ≈10%) with a fraction of the headcount (≈850 vs. ≈3,746 and ≈6,000). That's not a company winning through scale in the market-share sense — its market share is smaller — it's winning through operating leverage per unit of input. That pattern is hard to explain except through a systemic cost advantage, which is consistent with your activity-system-fit reading rather than, say, a single cheap-input explanation.

The capacity-utilization data reinforces this rather than standing apart from it: uneven utilization (40%–80% across labs) is exactly what you'd expect from a company whose advantage is the system's fixed-cost absorption, not any one facility's brilliance — there's real headroom to convert into further margin or share without rebuilding the model.

The strongest case against your interpretation:

Here's the tension you flagged yourself but haven't fully pressure-tested: if the advantage is really the system (multiple interlocking activities), that's a VRIN/activity-system-fit argument for durability. But you also separately raise founder-dependency as a real risk. Those two claims sit uneasily together. If Mr. B's personal cost discipline is a necessary ingredient of the system — not just a historical origin story but an ongoing input — then the advantage isn't fully organizational/systemic, it's partly person-dependent, which is a much weaker VRIN claim (rare and valuable, maybe, but not clearly non-substitutable or robust to succession). You can't fully have it both ways: either the system has been institutionalized independent of Mr. B (strong durability claim), or it still depends on him (weaker claim, and a real strategic risk the board should weight heavily). Right now your write-up asserts both without resolving the tension.

A second, sharper challenge: low switching costs and price transparency cut against durability, not just against your recommendation. You noted the industry has standardized tests, low switching costs, and increasing price transparency from online aggregators. If tests are commoditized and easy to compare, then a low-price strategy — even one built on a genuinely efficient cost base — may be more imitable at the price level than at the cost level. A well-funded competitor doesn't need to replicate your entire operating system to erode your advantage; it may only need to match your price while accepting a thinner margin, especially if it's optimizing for market share rather than profitability (which larger, more diversified competitors sometimes can afford to do longer than a smaller pure-play). This doesn't necessarily break your argument, but it does mean "hard to replicate the whole system quickly" and "hard to compete away the price advantage" are two different claims — and your current interpretation leans on the first without fully addressing the second.

Two questions, per your cap:

Does the case give you anything on how the two larger competitors are actually structured or resourced (e.g., their own processing model, degree of centralization, cost base) — enough to judge whether they could replicate the low-cost system if they chose to, or whether their much larger headcount/lower margins suggest a structurally different, harder-to-collapse cost base (e.g., hospital-embedded labs, different service mix)? This would let us judge imitability with evidence rather than assumption.

On the founder-dependency question specifically: does the case give you any detail on management depth below Mr. B — a leadership team, succession planning, delegated authority — or is he presented as the sole strategic decision-maker throughout? This is the fact that would let us judge whether the advantage is organizationally embedded or personally concentrated, which is the fork I flagged above.

Response to iteration 2

Competitors' structure and ability to imitate the model

The case gives enough information to show that the two larger competitors followed structurally different operating models, although it does not prove that they could not imitate the company's model.

One competitor operated approximately 359 laboratories, managed laboratories in more than 100 hospitals and employed around 6,000 people. It offered a broad range of pathology, molecular biology and microbiology tests and also had an imaging business. Hospital laboratory management was an important part of its model.

The other competitor had a national reference laboratory, 16 regional laboratories, around 190 satellite laboratories and 1,074 collection centres. It offered about 3,500 tests and employed more than 3,000 people.

In comparison, the company operated one central processing laboratory and six regional laboratories, with no satellite laboratories or company-owned collection centres. It employed about 850 people and relied on partners for sample collection.

These facts suggest that the competitors had more distributed and service-intensive cost structures. Their larger laboratory networks, broader test menus, hospital operations, imaging activities and higher employee numbers probably made it harder for them to reach the same cost per test without changing significant parts of their businesses.

However, I cannot conclude that they were incapable of imitation. They had larger revenues, established brands, wider test menus and access to capital. They could copy selected parts of the model, invest in automation or match the company's prices for strategic reasons. The case does not provide enough evidence about their internal capacity utilization, procurement contracts or willingness to accept lower margins.

My conclusion is therefore narrower: the competitors could probably imitate individual activities and customer prices, but replicating the company's complete cost structure would require substantial changes to their existing operating models. The case supports this as an interpretation, not as a proven fact.

Founder dependency and management depth

The case does show a management structure below the founder-CEO. The organization included managers responsible for business development, laboratory operations and infrastructure, along with a senior IT officer and a CFO. The organizational chart also shows deputy managers, a general manager for finance and a head of legal and secretarial functions.

The company employed around 850 people, many with technical expertise, and reportedly had low employee attrition. The founder-CEO also stated that a small team of senior managers, industry professionals and family members helped run the organization. His children and brother were involved in the business.

This means it would be inaccurate to describe him as the sole decision-maker or to claim that no management structure existed.

At the same time, the case does not describe a formal succession plan, an independent professional leadership pipeline or the extent to which major strategic decisions had been delegated. The founder-

CEO remained central to the company’s philosophy, operating choices and future direction. His personal emphasis on frugality appears repeatedly in the case.

I would therefore revise my interpretation as follows:

"The company had an established management team and experienced technical employees, so its operating advantage was not dependent on the founder-CEO alone. However, the case does not show whether its cost discipline and strategic judgment had been fully institutionalized beyond him. Founder dependence should therefore be treated as a governance and succession risk rather than as a confirmed weakness in the operating model."

This resolves the tension to some extent. The routines, technology and partner network were organizational assets, but the durability of the culture and strategic discipline after a leadership transition remains uncertain.

Iteration 3

AI response

Both revisions are careful and well-evidenced — you've resolved the tension I raised on founder dependency in exactly the right way: not by declaring it settled, but by separating what's institutionalized (routines, technology, partner network, a real org structure) from what remains genuinely unproven (whether cost discipline survives a leadership transition). That's a defensible, case-grounded position rather than a guess in either direction.

On competitor imitability, your narrower conclusion is the right calibration too — "could probably imitate individual activities and prices, but replicating the complete cost structure would require substantial changes to existing operating models" is a claim the case evidence actually supports, versus the stronger "cannot be imitated" claim you might have been tempted toward.

Final Conclusion –

1. How did the company disrupt the Indian diagnostic industry?

The company disrupted the industry by changing how diagnostic tests were collected and processed. Traditional diagnostic chains operated many laboratories and collection centres, carried larger workforces and offered thousands of tests. The company separated sample collection from testing. Independent partners handled the customer-facing collection work, while one central laboratory and six regional laboratories processed the samples.

This structure allowed the company to combine samples from across the country and process them in large volumes. Automation, standardized procedures, digital reporting and bulk purchasing reduced the cost per test. The company could therefore charge much lower prices than competitors while still earning a higher profit margin.

Its focus on preventive testing was also different from the industry's traditional dependence on sick-care testing. Preventive samples were less urgent and could be transported to centralized laboratories, making them particularly suitable for this model. By 2017, preventive and wellness testing contributed more than half of the company's revenue.

The disruption did not come from inventing a new diagnostic test. It came from redesigning the industry's value chain. The company used a partner-led front end and a centralized, automated back end to make diagnostic testing more affordable and scalable.

2. What was the company’s strategy?

The company followed a focused cost-leadership strategy.

It focused mainly on thyroid, biochemistry and preventive tests rather than matching the much wider test menus of larger competitors. Within this chosen segment, it aimed to become the lowest-cost processor by combining several connected activities:

- centralized sample processing
- automation and limited manual handling
- collection through independent partners
- high test volumes
- bulk purchasing of reagents and equipment
- digital reporting
- a lean organization and strict cost control

These activities reinforced one another. More collection partners generated more samples. Higher volumes improved laboratory utilization and purchasing power. Lower unit costs allowed the company to maintain low prices, which could attract still more customers and partners.

Its competitive advantage was therefore based on the fit between its activities, not on one machine or resource. Competitors could copy individual activities or match its prices for some time, but reproducing the full cost structure would require changes to their more distributed and employee-intensive operating models.

The company should continue strengthening this strategy by increasing the utilization of its existing laboratories and growing preventive testing. New businesses should be treated as controlled experiments. TB testing has a closer connection to the existing model, while PET-CT, water testing and owned wellness clinics require different capabilities and cost structures. Expansion in these areas should depend on clear evidence of demand, utilization and financial returns.

The company disrupted Indian diagnostics through business-model innovation rather than product innovation. Its focused cost-leadership strategy combined centralized processing, automation, partner-based collection and operational discipline to deliver tests at low prices while maintaining superior margins.

The most defensible course is to grow the core pathology and preventive-testing business first. Diversification should remain selective and should not weaken the cost structure that created the company’s advantage.